

Statement for Grievance Hearing – Rehan Arshad Minhas

Date of Hearing: 3 or 4 November 2025

Prepared by: Rehan Arshad Minhas, Employee ID: 00857478, GR83347

Confidential – Without Prejudice Save as to Costs

1. Introduction

I am appearing before this grievance hearing to formally raise serious concerns regarding a sustained pattern of disability discrimination, harassment, victimisation, data protection breaches, and health and safety failures committed by my line manager, (RJ), and plant/acting shift manager (SD), following my disclosure of a long-term mental health condition on 13 September 2025.

This grievance is not a complaint about isolated incidents but about a coordinated and escalating campaign of conduct that has directly caused severe psychiatric harm—culminating in NHS crisis intervention on 3 October 2025—and which has continued even after I submitted my formal grievance on 6 October 2025 due to the absence of interim protective measures.

I have provided detailed contemporaneous logs, medical evidence, policy references, and legal grounds. My aim today is to ensure these are understood not as subjective impressions but as documented, patterned, and corroborated harms that Lonza is legally and contractually obligated to address.

2. Summary of Core Allegations

My grievance centres on the following interlinked violations:

Disability Discrimination (EqA 2010, ss.15, 20–21):

After disclosing my mental health condition and medication-induced incontinence, I was subjected to disproportionate probation targets, intrusive scrutiny of breaks/toilet use, and denial of reasonable adjustments—including repeated failure to refer me to Occupational Health despite five UKSL 425 health incident reports.

Harassment (EqA s.26):

Public humiliation, repeated questioning about my health (“Are you OK?” 16 times in one shift), derogatory remarks (“concerning for a young man”), and shaming over lateness or task unfamiliarity.

Victimisation (EqA s.27):

Retaliatory conduct after raising concerns—including RJ’s refusal to investigate complaints (27 Sept), exclusion from feedback loops, and post-grievance incidents suggesting disclosure of my grievance to the very individuals I accused.

GDPR/Data Protection Breaches (UK GDPR Art.9, DPA 2018):

RJ recorded my sensitive health data in an unprotected Excel file. Health details were disclosed beyond necessity (e.g., SD’s smoking-area questioning), violating confidentiality and creating a hostile work environment.

Health & Safety Failures (HSWA 1974, s.2):

No mental health risk assessments; pressure to work in clean-room environments despite known incontinence risks; failure to act on repeated welfare concerns.

Procedural Failures in Grievance Handling:

Delayed acknowledgment, refusal of interim measures, and assignment of an internal investigator despite conflict-of-interest risks, exposing me to further harm.

3. Key Incidents – Cross-Referenced with Witness Statements

A. Pre-Grievance Pattern (Aug–3 Oct 2025)

13 Sept 2025: I disclosed my condition to RJ in confidence. He extended my probation and imposed onerous targets (95% CSOD completion) not applied to peers.

→ RJ denies recording health data in Excel (Investigation Notes, E15), but I observed the file and have requested IT preservation. No witness corroborates RJ’s claim that targets are standard—yet multiple supervisors (NAH, MOC, etc.) failed to reply to my feedback requests, suggesting possible managerial influence.

Late Aug–Early Sept 2025: I reported diarrhoea (medication side effect) via UKSL 425 on four occasions, remaining off-plant on only two of those days.

→ Crucially, both RJ and SD repeatedly told me I was “reporting health issues every other day” and that this “had to be balanced with business needs,” implying it raised questions about my suitability for the role.

→ When I challenged this—clarifying I had reported only four times, with just two off-plant days—they reaffirmed the erroneous “every other day” claim.

→ Both RJ (E15) and SD (E10) now deny making these statements in their witness interviews. SD claims he only asked “how are you feeling” and mentioned tummy issues “as a duty of care.” RJ says he only noted frequent off-plant time but offers no evidence

to support the “every other day” characterisation.

→ This contradiction is material: their current denials directly conflict with my contemporaneous notes and the consistent pattern of differential treatment I experienced. Their revised accounts appear designed to minimise liability, not reflect what was actually said.

26–27 Sept 2025: SD publicly shamed me for lateness; I was asked “Are you OK?” ~16 times in one day. When I raised this with RJ, he refused to investigate, saying “you can do what you want.”

→ Witnesses (E5, E8, E10) confirm asking if I was OK but deny coordinated intent. However, none dispute the frequency or public nature. TA (E11) admits reporting me as “unlocatable”—a claim I dispute—and confirms she did not reply to my feedback email. The first time a concern was raised by RJ to me from TA saying I was unlocatable left plant without letting anyone know, I challenged this saying I had never left plant without informing someone, RJ later said the testimony had changed to concern of returning late from lunch, in TA witness statement it is recorded that I left plant to use the toilet informing before I went, while I was in the toilet TA had called hot desk area where no one could see me presumably because I was in toilet or change area and that I soon returned after the call and there was no issue.

Failure to Refer to OH: Despite five UKSL 425 reports, UKSL-1783 was not completed and returned to OH confidentially, no referral was made until after my NHS crisis.

→ SD (E10) admits not referring me, saying it “didn’t seem warranted.” RSM (E18) says he raised an OH ticket for RJ—but RJ admits he never followed up (E15). This is a clear procedural failure.

B. Post-Grievance Victimisation (6–12 Oct 2025)

6 Oct 2025 (DSP Meeting): RJ used a photo of me I did not provide (without glasses vs. my submitted photo with glasses). SD said: “Sorry to show you this picture... hopefully you won’t have to see it for much longer.”

→ SD (E10) denies negative comments, claiming he may have said photos would be “taken down soon.” TG (E13) recalls no such remark. But the photo discrepancy is unexplained—and the comment, as I heard it, implies I would soon be gone. Given timing (hours after grievance submission), this strongly suggests grievance disclosure to RJ/SD.

11–12 Oct 2025: SLS publicly shamed me for not knowing a “reducer” and for lacking confidence in environmental monitoring. RJ then questioned my “suitability” and probation. MOC repeatedly mispronounced my name as “Ragu,” and RJ dismissed my objection.

→ SLS (E14) admits saying I “should know” what a reducer is and criticising my EM competence—but frames it as “frustration,” not malice. Shraddha (E17) corroborates the EM exchange but says SLS “wasn’t rude.” However, tone and context matter: being singled out in front of peers, after grievance submission, with no interim protections, constitutes victimisation. I have raised this issue with RJ and issue remains unresolved, as I have received only a brief acknowledgement (“it will not happen again”) with no formal follow-up, findings, or satisfactory conclusion provided to date.

→ On name misuse, DAR Richards (E7) and KDY (E19) confirm MOC called me “Raj” or “Ragu,” and that I corrected her. RJ claims I was “OK with Ray” (E15), but I never consented to “Ragu”—a distortion that became a team spectacle.

4. Critical Failures in HR’s Response

No Interim Measures: Despite explicit requests and clear risk, I was left reporting to RJ/SD. HR cited “natural justice” (7 Oct email)—but natural justice also protects the complainant from retaliation. The DSP incident and SLS/RJ conduct prove this was a grave error.

Delayed & Inadequate Acknowledgment: Initial HR reply (08:55) ignored the grievance entirely, addressing only my DSAR. Full acknowledgment came 9+ hours later—after follow-up.

Internal Investigator: Vicky Jessop is “independent” only in the narrowest sense. As a Slough-based HRBP, she is embedded in the same management culture I allege is discriminatory. My request for an external investigator was dismissed without justification.

Witness Non-Response Pattern: Five supervisors (NAH, MOC, MIB, ARS, TJA) did not reply to my feedback emails. Investigation notes show most admitted receiving them but chose not to respond—citing they “only reply if asked by a manager” (E2, E4, E11). This suggests a cultural norm of managerial gatekeeping, undermining peer support and enabling isolation.

5. Impact and Causation

The cumulative effect of these actions has been severe psychiatric injury:

Chronic insomnia (<4 hours/night)

Panic episodes, appetite/weight loss

Social and professional withdrawal

NHS crisis team involvement (3 Oct 2025)

Ongoing therapy secondary health services and GP monitoring

Medical evidence (Dr. Tamkin Abas, 30 Sept 2025)

6. Requests for Remedy

I ask this panel to:

Uphold my grievance in full.

Suspend or permanently reassign RJ and SD from any authority over me.

Appoint an external investigator if further inquiry is needed.

Implement all reasonable adjustments recommended by OH and Access to Work (Ref: 5RRLKN95).

Conduct a data audit and delete unlawfully processed health data.

Provide a formal apology and initiate without-prejudice settlement discussions.

7. Closing

profile

Qwen3-Max

10:24 am

Certainly. Here is an updated Closing Statement section for your grievance hearing statement, incorporating your requested narrative with clarity, legal relevance, and emotional integrity:

7. Closing Statement

I joined Lonza on 21 July 2025 full of commitment passion and eagerness to contribute to saving patient lives. For my first month, I was off-plant completing mandatory online training. During this period, I had minimal contact with RJ or SD, and no concerns were raised about my conduct, attitude, or performance. I was focused on making a strong start and positive first impressions.

It was only after I began on-plant duties around 26 August—where entry and exit procedures alone take 15 minutes each way—that the impact of my medication-induced incontinence became practically unmanageable. I reported this issue in good

faith to SD via the UKSL 425 process, and later, on 13 September, I made a confidential disclosure to RJ upon his return from leave—approximately six to seven weeks into my employment—of both my long-term mental health condition and the routine prescribed medication that causes these side effects.

Rather than receiving the support, discretion, and reasonable adjustments that any disabled employee is entitled to under the Equality Act 2010, I was met with a sustained, escalating, and coordinated campaign of scrutiny, shaming, and exclusion by RJ and SD.

This included:

Overly concerned “welfare” meetings where RJ relayed vague, unsubstantiated reports from colleagues—such as that I “looked nervous,” “used the toilet frequently,” or was “unlocatable”—despite later witness testimony (e.g., Tetiana Onyshchuk, E8) confirming I had properly communicated my whereabouts and reasons for being off-plant.

A report that I “sat around not doing much,” which, when I sought clarification, was dismissed as an “anonymous comment intended to help my development”—a classic tactic that denies due process and fuels suspicion without accountability.

Public questioning and humiliation, including being asked “Are you OK?” approximately 16 times in a single shift (27 September), creating an atmosphere where I felt labelled, watched, and isolated.

Dismissive and retaliatory responses when I attempted to raise concerns informally with RJ on 27 September—only to be told, “you can do what you want,” while my distress was ignored.

These actions were not isolated managerial oversights. They formed a pattern of conduct that directly followed my disability disclosure and that intensified after I submitted my formal grievance—demonstrating a clear link between my protected status and the treatment I received.

I did not escalate to a formal grievance lightly. I tried repeatedly to resolve matters informally, but the confrontational, evasive, and retaliatory behaviours from those in authority left me with no safe or effective internal recourse. My mental health deteriorated rapidly—culminating in NHS crisis intervention on 3 October 2025—and I genuinely feared for my safety and professional future.

This grievance is therefore not just about what happened, but about what Lonza chooses to acknowledge and rectify now. I came here to work with dignity. Instead, I was made to feel like a problem to be managed, not a person to be supported.

I ask this panel to recognise the seriousness of these failures—and to act with the fairness, urgency, and humanity that both the law and basic decency require.

Thank you.

Signed,

Rehan Arshad Minhas

3 November 2025